

Your First 30 Days

A printable action plan from *Osteoporosis: The Book I Wish I'd Had When I Was Diagnosed*. Chapter 12, set out as a checklist you can work through at your own pace. Nothing here is medical advice, and your own clinician may want you to do things in a different order.

Before you start

Finding out you have osteoporosis or osteopenia can feel overwhelming.

You might be wondering where to start, what to do first, and how to prioritize the many recommendations you're receiving.

This action plan will guide you through your first month, giving you concrete steps to take while you're still processing the diagnosis.

Important note: This timeline assumes you were diagnosed through screening (a DEXA scan without recent fractures).

If you're reading this because you recently fractured a bone, your first priority is healing.

Skip ahead to the "If You're Recovering from a Fracture" section for modified guidance that prioritizes recovery while still taking important first steps for your bone health.

Week 1: Information Gathering and Assessment

Days 1–3: Understand Your Diagnosis

- ☐ Request a copy of your complete DEXA scan results, including T-scores for all tested sites (spine, hip, forearm).
- ☐ Ask for your Z-score as well. If available, request your FRAX or FRAXplus score.
- ☐ Ask about TBS if it wasn't included.
- ☐ Write down any questions that come up as you review your results.

Days 4–7: Schedule Essential Appointments

- ☐ Make an appointment with your primary care doctor or endocrinologist to discuss treatment options (aim to do this within two to four weeks).
- ☐ Gather previous results if you've had relevant tests done (vitamin D level, calcium level, parathyroid hormone, thyroid function, complete blood count, kidney and liver function, bone turnover markers).
- ☐ If you haven't seen a dentist in the past year, schedule a dental checkup (important before starting certain bone medications).

Throughout Week 1: Start a Bone Health Journal

- ☐ Record your current height (measure against a wall).
- ☐ List all medications and supplements you're currently taking.
- ☐ Note any symptoms you've been experiencing (back pain, height loss, etc.).
- ☐ Document your family history of osteoporosis or fractures.
- ☐ Track questions to ask your doctor.

Week 2: Begin Safe Lifestyle Changes

Days 8–10: Assess Your Environment

- ☐ Walk through your home looking for fall hazards (refer to Chapter 11).
- ☐ Make a list of safety improvements needed.
- ☐ Start with quick fixes: remove throw rugs, improve lighting, clear clutter from walkways.
- ☐ Check that you have sturdy handrails where you need them.

Days 11–14: Start Gentle Movement

- ☐ If you aren't doing so already, begin walking daily, even if just for 10–15 minutes.
- ☐ Practice standing on one foot while holding on to a counter (work up to 30 seconds per side).
- ☐ Start noticing your posture throughout the day; aim to keep your tummy in and shoulders back.
- ☐ Avoid forward bending, twisting, or high-impact activities until you discuss exercise with your healthcare provider.

Throughout Week 2: Dietary Assessment

- ☐ Track what you eat for three to four days.
- ☐ Calculate your approximate daily calcium intake (see Chapter 8).
- ☐ Note how much caffeine, alcohol, and soda you typically consume.
- ☐ Identify one dietary change you can make this week (example: swap afternoon soda with water, or swap potato chips with almonds).

Week 3: Treatment Planning and Implementation

Days 15–17: Medical Appointments and Decisions

- ☐ Attend your doctor's appointment with your list of questions.
- ☐ First, discuss whether you need screening for other medical conditions that could be causing bone loss.
- ☐ Review your health history and medication list for possible "hidden thieves" (see Chapter 6).
- ☐ Expect, at minimum, some basic blood tests, such as a metabolic panel and a Vitamin D level to be ordered.
- ☐ Your healthcare provider should also consider tests for: thyroid or parathyroid disease, inflammatory disease, Celiac disease, diabetes, Cushing's disease, and multiple myeloma.
- ☐ After testing is done and any abnormal results addressed, ask about osteoporosis treatment: Is medication recommended for your situation?
- ☐ Could you try diet and exercise first, and if so, when will a follow-up DEXA be done?
- ☐ Discuss when to start, what side effects to watch for, how long you'll need to take the medication, and the monitoring schedule.

Days 18–21: Supplement Strategy

- ☐ Based on your bloodwork results, start appropriate supplements: calcium citrate (if dietary intake is insufficient), vitamin D (dosage based on your blood level), vitamin K2-MK7 (if recommended by your doctor), and magnesium (if not getting enough from diet).
- ☐ Consider collagen peptides (discuss with your doctor).
- ☐ Set up a system to remember daily supplements (phone alarm, pill organizer).
- ☐ Take calcium in divided doses (no more than 500 mg at once).
- ☐ Take calcium and vitamin D with food for better absorption.

Throughout Week 3: Medication Education

- ☐ If you're prescribed medication, read all patient information carefully and consult with your pharmacist.
- ☐ Note the best time to take it. Understand what to do if you miss a dose.
- ☐ Know what side effects warrant calling your doctor.
- ☐ Set up reminders to take medication correctly.

Week 4: Building Your Support System and Long-Term Plan

Days 22–24: Assemble Your Healthcare Team

- ☐ Identify who will manage your osteoporosis care (primary care doctor, endocrinologist, or another specialist).
- ☐ Ask for referrals if needed: physical therapist with osteoporosis expertise, registered dietitian, endocrinologist or rheumatologist with osteoporosis expertise.
- ☐ Schedule your next follow-up appointment (typically in three to six months).

Days 25–28: Exercise Program Development

- ☐ If possible, schedule an appointment with a physical therapist who specializes in osteoporosis.
- ☐ Discuss which exercises are safe and beneficial for your specific situation.
- ☐ Get guidance on proper form and progression.
- ☐ Create a realistic weekly exercise schedule (refer to Chapter 7).
- ☐ Start with two to three days of strength training and daily walking per week.

Days 29–30: Create Your Ongoing Management Plan

- ☐ Review what you've accomplished in the past month, and recalibrate your efforts as needed.
- ☐ Set three-month goals: exercise consistency target, dietary improvements, weight management goals (if applicable), fall-prevention measures completed.
- ☐ Mark your calendar for the following: next DEXA scan (typically in one to two years, but your doctor will advise), next medical appointment, regular medication refills, bloodwork follow-ups.

End of Month: Celebrate Your Progress

Take a moment to acknowledge that you've taken control of your bone health.

You've moved from shock and uncertainty to active management.

This is a significant accomplishment.

If You're Recovering from a Fracture

Your first 30 days look different, because healing takes priority. Here's your modified plan.

Week 1: Focus on Healing and Pain Management

- ☐ Follow your doctor's instructions for activity restrictions carefully.
- ☐ Take prescribed pain medication as directed; contact a pain management doctor if your pain is severe.
- ☐ Get adequate rest. Make sure you're eating enough protein and calories for healing.
- ☐ Start calcium and vitamin D supplementation (if not already taking).
- ☐ Ask about testing for other reasons for weakening of the bones.
- ☐ If you're younger (under 65) or have any history of cancer, it's important to ask your doctor whether more serious conditions (like cancer or multiple myeloma) could be causing your fractures.
- ☐ What to skip: Don't start an exercise program yet.
- ☐ Don't attempt balance exercises or strength training.
- ☐ Avoid assessing your home for fall hazards on your own (you may not be steady enough).

Week 2: Gentle Resumption of Activity

- ☐ Begin whatever gentle movement your doctor has cleared (this may be limited initially; you may be told to get moving right away or to rest as much as possible).
- ☐ Work with physical therapy if prescribed.
- ☐ Continue focusing on nutrition and healing.
- ☐ Start a bone health journal when you feel up to it.
- ☐ Don't rush recovery. Avoid any exercises that cause pain.

Week 3: Planning for Long-Term Management

- ☐ Schedule or attend appointments to discuss osteoporosis treatment.
- ☐ Begin to address one dietary change if you're feeling better.
- ☐ Review medications with your doctor (some may have contributed to your fracture risk).
- ☐ Ask about the timeline for more active bone-strengthening exercises.
- ☐ Don't be surprised if you start feeling down or discouraged; find someone to talk to about your feelings.

Week 4: Building Your Foundation

- ☐ If you're healing well, begin to implement home safety changes (with help).
- ☐ Start planning your exercise program for when you're fully healed.
- ☐ Focus on consistency with supplements and any prescribed medications.
- ☐ Set realistic expectations for your recovery timeline.

Important Reminders for Fracture Recovery

Healing varies widely by individual and fracture type.

Single vertebral fractures may heal in 6–12 weeks, but multiple vertebral fractures can take much longer (sometimes a year or more).

Hip fractures typically require three to six months to heal.

Ask your doctor about scheduling follow-up imaging to monitor healing progress.

Pain should gradually improve week by week, though the timeline varies, and you can expect pain to rebound a little bit when you start new activities.

Your activity level will increase slowly under medical supervision.

Some of the recommendations in this book (especially Chapter 7 on exercise) should be postponed until you're healed.

Be patient with yourself; recovering from a fracture while processing an osteoporosis diagnosis is genuinely difficult.

Beyond the First 30 Days

By the end of your first month, you've established the foundation.

Now, the focus shifts to maintaining consistency with medications and supplements, gradually building your exercise program, making ongoing dietary improvements, attending follow-up appointments, monitoring your progress, and staying educated about updates in bone health.

Remember, managing osteoporosis is a marathon, not a sprint.

The actions you take in these first 30 days set the stage for long-term bone health, but give yourself grace as you adapt to this new reality.

Some days will feel overwhelming. Other days, you'll feel empowered and in control. Both responses are normal.

You're not alone on this journey, and taking action, even small steps, makes a real difference in your bone health outcomes.

Keep moving forward, one day at a time.

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